

# ACUTE APPENDICITIS — Clinical Revision Table

*Bold = high-yield. Commonest cause of acute abdomen / surgical admission (UK). Peak 10–20 yrs.*

## 1 — DEFINITION, EPIDEMIOLOGY & PATHOGENESIS

|                  |                                                                                                                                                                                                                                                                                 |
|------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Definition / epi | Inflammation of vermiform appendix. <b>Commonest acute abdomen + surgical admission (UK); ~1 in 7.</b> Peak <b>10–20 yrs</b> , rare <3 yrs                                                                                                                                      |
| Pathogenesis     | <b>Mechanical (main): luminal obstruction (faecolith, kinking, foreign body)</b> → luminal hypertension → vessel compression → venous/lymphatic block → wall oedema → thrombosis → mucosal barrier loss → gut flora invade wall → mural destruction. Also vascular + infectious |

## 2 — CLINICAL FEATURES

|                   |                                                                                                                                                                                       |
|-------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Pain              | <b>Vague colicky CENTRAL (periumbilical) visceral pain</b> → <b>localizes to RIF, becomes constant</b> (parietal involvement). Classic migration. + low-grade fever, N/V, anorexia    |
| Position variants | Pelvic → urinary symptoms/diarrhoea. Radiation: perineum (pelvic), right lumbar (retroperitoneal), right hypochondrium (retrocaecal), mesogastrium (median). 'Monkey of all diseases' |
| Sepsis signs      | Low-grade pyrexia → <b>spikes 38–39°C if perforation/abscess</b> ; tachycardia, flushing, dehydration                                                                                 |

## 3 — EXAMINATION SIGNS

|                  |                                                                                                                                            |
|------------------|--------------------------------------------------------------------------------------------------------------------------------------------|
| McBurney's point | Tenderness = usual feature. Peritoneal signs: guarding, percussion tenderness, pain on cough/movement; rigidity if generalized peritonitis |
| Rovsing's sign   | Pressure on LIF → pain in RIF (needs RIF tenderness too)                                                                                   |
| Psoas sign       | Hip held flexed to relieve pain (appendix adjacent to psoas)                                                                               |
| PR exam          | Tenderness anterolaterally on right                                                                                                        |

## 4 — DIAGNOSIS & ALVARADO SCORE

| Diagnosis            | <b>Clinical primarily.</b> FBC (leucocytosis), urinalysis (exclude UTI), <b>pregnancy test MANDATORY (ectopic)</b> , US (women → tubo-ovarian), CT (elderly caecal tumour/obese), diagnostic laparoscopy, U&E |     |     |
|----------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----|-----|
| Alvarado feature     | Category                                                                                                                                                                                                      | Pts |     |
| Migration of pain    | Symptom                                                                                                                                                                                                       | 1   |     |
| Anorexia             | Symptom                                                                                                                                                                                                       | 1   |     |
| Nausea/vomiting      | Symptom                                                                                                                                                                                                       | 1   |     |
| RIF (RLQ) tenderness | Sign                                                                                                                                                                                                          | 2   |     |
| Rebound              | Sign                                                                                                                                                                                                          | 1   |     |
| Elevated temperature | Sign                                                                                                                                                                                                          | 1   |     |
| Leukocytosis         | Lab                                                                                                                                                                                                           | 2   |     |
| Left shift           | Lab                                                                                                                                                                                                           | 1   |     |
| INTERPRETATION       | <b>7–10 strongly predictive · 5–6 equivocal (→imaging) · 1–4 ruled out</b>                                                                                                                                    |     | /10 |

## 5 — MANAGEMENT, INDICATIONS & COMPLICATIONS

|                         |                                                                                                                                                                                                                                                                 |
|-------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Management              | Resuscitate (IV fluids, NPO), analgesics, antibiotics, <b>appendicectomy within 24h ASAP. Open</b> (children, Lanz incision, lavage if perforated). <b>Laparoscopic</b> (young women, obese; ↓wound infection, ↓stay, faster recovery) — preferred if expertise |
| Appendix mass (late)    | CT → check abscess/caecal tumour. <b>Initial conservative</b> (IV fluids, antibiotics, observe ± drainage). Surgery if deterioration/perforation                                                                                                                |
| Indications for surgery | Acute appendicitis, recurrent appendicitis, <b>mucocoele, carcinoma</b>                                                                                                                                                                                         |
| Special                 | <b>Recurrent:</b> incomplete self-limiting obstruction. <b>Pregnancy:</b> most common extrauterine acute abdomen (0.5–1/1000)                                                                                                                                   |
| Complications           | <b>Abscess (peri-appendicular/pelvic/sub-hepatic)</b> , post-op collection, wound infection/haematoma, intestinal obstruction (adhesions)                                                                                                                       |